Eating Disorders (DSM-5-TR): Teaching Overview

Clinical Companion Knowledge Library: for counselors and counseling students

1. What it is

Eating disorders are persistent disturbances in eating and eating-related behavior that harm physical health or functioning. DSM-5-TR's chapter "Feeding and Eating Disorders" includes anorexia nervosa, bulimia nervosa, binge-eating disorder, and avoidant/restrictive food intake disorder (American Psychiatric Association [APA], 2022). In anorexia and bulimia, self-worth depends heavily on weight and shape.

How common: in a U.S. national sample, lifetime prevalence was 0.80% for anorexia nervosa, 0.28% for bulimia nervosa, and 0.85% for binge-eating disorder, all higher in women (Udo & Grilo, 2018). Across 94 studies worldwide, 8.4% of women and 2.2% of men had a lifetime eating disorder (Galmiche et al., 2019).

Among the deadliest mental disorders: across 36 studies, death rates were nearly 6 times expected in anorexia nervosa (standardized mortality ratio 5.86) and about twice expected in bulimia nervosa (1.93); one in five deaths in anorexia was a suicide (Arcelus et al., 2011).

Counselor framing: eating disorders are serious medical-psychiatric illnesses, not choices or vanity. Medical monitoring comes first; counseling works alongside it.

2. Core symptoms to watch for

DSM-5-TR criteria, paraphrased (APA, 2022); use the manual itself for formal diagnosis.

- **Anorexia nervosa:** restricting food intake to a significantly low body weight; intense fear of gaining weight (or persistent behavior that prevents weight gain); and a disturbed view of one's body or failure to recognize the seriousness of low weight. Subtypes: restricting, or binge-eating/purging.
- **Bulimia nervosa:** recurrent binge eating (a large amount with loss of control) and recurrent compensatory behavior (vomiting, laxatives, fasting, excessive exercise), both at least weekly for 3 months, with self-evaluation unduly shaped by weight and shape, at normal or higher weight.
- **Binge-eating disorder:** recurrent binges at least weekly for 3 months, with marked distress and 3 or more of: eating rapidly, until uncomfortably full, large amounts when not hungry, alone out of embarrassment, and feeling disgusted, depressed, or guilty afterward; no regular compensatory behavior.
- **Avoidant/restrictive food intake disorder:** restricted eating that causes weight loss, nutritional deficiency, or impairment, driven by low interest, sensory aversion, or fear of consequences (such as choking) rather than body-image concerns.

Screening: the 5-question SCOFF was developed as a brief screening tool for eating disorders (Morgan et al., 1999). Ask directly about restricting, bingeing, purging, and compensatory exercise; many people hide these.

Suicide risk: suicide is the second leading cause of death in anorexia nervosa, and roughly one-quarter to one-third of people with anorexia, bulimia, or binge-eating disorder have thought about suicide (Smith et al., 2018).

When counselors must refer (general clinical consensus): every client needs a medical evaluation. Refer urgently for very low or rapidly falling weight, fainting, chest pain, frequent purging, or suicidality. Work in a team with a physician and a dietitian.

3. What works: treatment and the evidence

Bottom line for class: CBT is the first-line treatment for bulimia nervosa and binge-eating disorder. For adolescents with anorexia nervosa, family-based treatment has the best evidence. Adult anorexia remains the hardest to treat, and several therapies work about equally well.

Evidence overview (honest findings):

- **Bulimia and binge-eating disorder:** therapist-led CBT beat wait-lists and other psychotherapies, was most effective in its manualized and enhanced forms, matched antidepressants at the end of treatment, and outperformed

interpersonal therapy on symptoms after treatment (Linardon et al., 2017). CBT has kept its status as first-line therapy for bulimia (Hay, 2013).

- **Transdiagnostic CBT** (CBT-E) treats the shared core of all eating disorders (Fairburn et al., 2003). In a trial with 154 outpatients who were not markedly underweight, it produced large, lasting improvement, and 51.3% were near the community norm at 60 weeks, regardless of diagnosis (Fairburn et al., 2009).
- **Binge-eating disorder:** psychotherapy, mostly CBT, had large effects on binge eating, and CBT-based self-help had medium-to-large effects; medications had small effects (Hilbert et al., 2019). Lisdexamfetamine at 50 to 70 mg reduced binge days compared with placebo (McElroy et al., 2015).
- **Adolescent anorexia:** family-based treatment produced more full remission than individual therapy at 6 and 12 months (Lock et al., 2010).
- **Adult anorexia:** among 242 outpatients, enhanced CBT, focal psychodynamic therapy, and optimized usual care all increased weight, with no differences between them at 12 months (Zipfel et al., 2014).

CBT for eating disorders: how it is used. The core problem is an over-evaluation of shape, weight, and their control, which drives strict dieting; dieting sets up binges, and binges lead to compensatory behaviors (Fairburn et al., 2003). Treatment therefore:

- **Establishes regular eating** (three meals and planned snacks) to break the diet–binge cycle, with weekly weighing done together (Fairburn et al., 2003).
- **Identifies and challenges** beliefs that tie worth to weight: "I'm only acceptable if I'm thin," "If I eat this, I'll lose control," "My body is disgusting" (Fairburn et al., 2003).
- **Addresses** body checking and avoidance, perfectionism, low self-esteem, and mood triggers (Fairburn et al., 2009).

4. Religion, spirituality, and Christian integration: what the evidence shows

A secure relationship with God appears protective. Across 22 studies, deep, internalized faith and a secure relationship with God were linked to less disordered eating and body-image concern, while a superficial faith and an anxious relationship with God were linked to more (Akrawi et al., 2015).

Spiritual growth during treatment. Among 251 women in inpatient treatment, religiousness itself did not predict outcomes, but gains in spiritual well-being during treatment went with better eating attitudes, less body-shape concern, and better functioning (Smith et al., 2003).

Intervention studies (few, but promising):

- **Spirituality group** (randomized, 122 women inpatients): compared with cognitive or emotional-support groups, women in a spirituality group had lower psychological disturbance and eating-disorder symptoms at discharge and improved faster in the first 4 weeks (Richards et al., 2006).
- **Christian body affirmations** (randomized, college women, mostly Christian): reading affirmations of God's love and acceptance of their bodies significantly improved how women felt about their appearance after viewing thin-ideal images, compared with a control group; secular affirmations helped only marginally (Boyatzis et al., 2007).

Honest summary for teaching:

1. CBT (including CBT-E), family-based treatment for adolescents, and medical monitoring are the evidence base. Faith integration supports them; it does not replace them.
2. A secure, internalized faith is linked to less disordered eating, and spiritual growth during treatment tracks recovery.
3. Faith-based interventions show early promise in small randomized studies, but no trial yet tests a full Christian-integrated CBT for eating disorders.

5. Practical Christian-integration notes

These apply Dr. Campbell's identify, challenge, replace-with-Scripture method within CBT and medical care. Clinical items rest on the research above. Pastoral items are theological application, not research findings.

- **Identify (clinical).** Explore how the person believes God sees them and their body. Listen for "God is disgusted with me when I eat," "Thinness is self-control and godliness," or "My body is sinful." Ask whether religious fasting is being used to hide restriction (general clinical consensus).
- **Challenge (clinical).** Test the belief that worth depends on weight or shape, and weigh "control" against what the disorder is actually doing to health, relationships, and faith (Fairburn et al., 2003). Gently address an anxious, punitive image of God (Akrawi et al., 2015).

- **Replace (pastoral).** Offer Scripture of God's acceptance and of the body's goodness:
  - "I praise You, for I am fearfully and wonderfully made. Marvelous are Your works, and I know this very well." (Psalm 139:14)
  - "…the LORD does not see as man does. For man sees the outward appearance, but the LORD sees the heart." (1 Samuel 16:7)
  - "For every creation of God is good, and nothing that is received with thanksgiving should be rejected." (1 Timothy 4:4). Food received with thanks is a gift, not an enemy.
  - "Do you not know that your body is a temple of the Holy Spirit who is in you…? Therefore glorify God with your body." (1 Corinthians 6:19–20). Caring for the body, including eating, honors God.
- **Faith community (clinical).** With consent, help family and church avoid diet talk and body comments, and support regular meals. Consider fasting practices carefully during recovery (general clinical consensus).
- **Ministers:** never encourage weight loss or fasting in someone you suspect has an eating disorder; express concern kindly and refer for medical evaluation.

6. Quick classroom takeaways

- Anorexia nervosa has the highest death rate of the eating disorders, and suicide accounts for many deaths. Screen (SCOFF), ask directly, and always involve a physician.
- CBT and CBT-E are first-line for bulimia and binge-eating disorder; family-based treatment for adolescent anorexia; adult anorexia needs a team approach.
- A secure, internalized faith is protective. Challenge beliefs that tie worth or holiness to weight, and replace them with the truth that God made the body good and sees the heart.

Sources note

All 18 references were checked against Crossref and PubMed (Oct 31, 2026), and the figures quoted were checked against each study's abstract. Each DOI is a live link to the publisher record.

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